BASELINE → TREATMENT → FOLLOW-UP

peptide receptor radionuclide therapy (PRRT) tests before, between and after four intended administrations

This is the National Cancer Control Programme (NCCP) blood-test sequence with EMA potassium safety and the PRRT service pathway. The post-infusion radiation-assessment scan is different from a response scan. The service confirms bookings, exact dates and any additional follow-up.

Check	Before the course	Before each administration	Between administrations	After the final dose	neuroendocrine tumour (NET) response / safety plan
Blood count / kidneys / liver	Baseline tests	Once 2–4 weeks beforehand AND shortly before infusion	Every 2 weeks	Every 4 weeks for at least 3 months, then every 3 months for the following year	Additional tests or longer follow-up if abnormal
Potassium / renal clearance	Assess renal function and electrolyte risks	Potassium before amino acids; renal function and suitability reviewed	Renal checks during therapy; earlier assessment if ill	Renal monitoring for at least the first year	Fluid and electrolyte plan individualised
Heart / pregnancy / hormones	echocardiogram (heart ultrasound) (ECHO) or multigated acquisition (heart pumping) scan (MUGA); pregnancy testing where applicable; hormone plan	Review symptoms and any pregnancy possibility; somatostatin analogue (SSA) plan confirmed	Report hormone symptoms, especially during first 48 hours	Follow hormone symptoms and contraception plan	Additional heart tests if clinically indicated
Imaging / clinic	somatostatin receptor (SSTR)-positive disease and baseline assessment	Specialist clearance for each treatment	PRRT clinic review; special scan(s) after each infusion for radiation assessment	Response and follow-up imaging as agreed	No universal mandatory tumour-marker or response positron emission tomography (PET) interval
Radiation precautions	Discuss household, travel, work and care needs	Receive written instructions for this administration	Observe each restriction until its own end date	Keep discharge information and tell other treating teams	Medical physicist confirms personal restrictions

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00642 v2 · Lutathera regimen · EMA Lutathera product information and patient leaflet · updated 14 August 2026 · HSE / PRRT service: introductory patient information

Tests in your plan

Full blood count

WHEN

Before treatment; once 2–4 weeks before and shortly before each administration; every 2 weeks between treatments.

Checks haemoglobin, white cells with differential, and platelets.

After the last administration: every 4 weeks for at least 3 months, then every 3 months for the following year. Your written plan may include extra early checks.

Kidney function

WHEN

With the blood-test schedule above; monitor for at least the first year after treatment.

Serum creatinine and calculated creatinine clearance assess kidney safety.

The team calculates clearance using the required method. A normal symptom check does not replace these tests.

Liver function

WHEN

With the blood-test schedule above.

Tests include alanine aminotransferase (ALT), aspartate aminotransferase (AST), albumin, bilirubin and international normalised ratio (blood-clotting test) (INR) to assess liver function and protein production.

Potassium

WHEN

Before the kidney-protective amino-acid infusion; further checks if indicated.

Detects potassium disturbances that the amino-acid infusion can worsen.

Heart pumping assessment

WHEN

Before starting, as specified in the regimen.

An echocardiogram or MUGA scan assesses heart pumping function.

5-hydroxyindoleacetic acid (5-HIAA)

If part of your plan

WHEN

Only if requested for your hormone syndrome; the team sets the dates.

Measures a breakdown product of serotonin to help assess selected neuroendocrine tumours.

Often a timed urine collection. Follow the laboratory's food and medicine instructions.

Chromogranin A

If part of your plan

WHEN

Only when the team considers it informative for your tumour.

A selected tumour-marker result, interpreted with symptoms and scans.

Acid-suppressing medicines and other conditions can alter the result. It is not a routine test before every treatment; do not stop medicines yourself.

Selected pancreatic hormones

If part of your plan

WHEN

Only for the hormone question identified by your specialist.

Examples include gastrin, insulin or glucagon when the diagnosis or symptoms call for them.

The team names the test, preparation and review date; these are not a standard panel for every patient.

THE WHOLE TREATMENT JOURNEY

Lutetium-177 oxodotreotide (Lutathera / PRRT): treatment and checks

Progressive, unresectable or metastatic, well-differentiated G1/G2 somatostatin-receptor-positive gastroenteropancreatic NET under NCCP 00642a.

Four administrations are planned, usually eight weeks apart. Lutathera is infused over about 30–40 minutes, with a four-hour kidney-protective amino-acid infusion and anti-sickness treatment. The visit is much longer than the Lutathera infusion; an overnight stay may be needed.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before the course	Before each administration	Between administrations	After the final dose	NET response / safety plan
Blood count / kidneys / liver NCCP 00642	Baseline tests	Once 2–4 weeks beforehand AND shortly before infusion	Every 2 weeks	Every 4 weeks for at least 3 months, then every 3 months for the following year	Additional tests or longer follow-up if abnormal
Potassium / renal clearance NCCP 00642	Assess renal function and electrolyte risks	Potassium before amino acids; renal function and suitability reviewed	Renal checks during therapy; earlier assessment if ill	Renal monitoring for at least the first year	Fluid and electrolyte plan individualised
Heart / pregnancy / hormones NCCP 00642	ECHO or MUGA; pregnancy testing where applicable; hormone plan	Review symptoms and any pregnancy possibility; SSA plan confirmed	Report hormone symptoms, especially during first 48 hours	Follow hormone symptoms and contraception plan	Additional heart tests if clinically indicated
Imaging / clinic NCCP 00642	SSTR-positive disease and baseline assessment	Specialist clearance for each treatment	PRRT clinic review; special scan(s) after each infusion for radiation assessment	Response and follow-up imaging as agreed	No universal mandatory tumour-marker or response PET interval
Radiation precautions NCCP 00642	Discuss household, travel, work and care needs	Receive written instructions for this administration	Observe each restriction until its own end date	Keep discharge information and tell other treating teams	Medical physicist confirms personal restrictions

What each check is for

1

Four planned PRRT administrations

T1–T4

PRRT carries radiation to somatostatin receptors on NET cells to help control disease that cannot be removed or has spread. A specialist NET and nuclear-medicine team checks receptor imaging, organ function and your priorities before recommending it.

Ask / **act:** Confirm the current prescription, tests and support medicines.

Sources: NCCP 00642

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-05 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.37